

Patient Name : MR. KOTHURU JESHWANTH**Age / Gender** : 21 years / Male**Patient ID** :396858**Referral** : Dr. SELF**Sample Type** : Edta Wb**Source** : DoctorC**Collection Time** : Sep 06, 2026, 03:30 p.m.**Receiving Time** : Sep 06, 2026, 05:48 p.m.**Reporting Time** : Sep 06, 2026, 06:15 p.m.**Sample ID** :

8276211

HAEMATOLOGY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
Complete Blood Picture (CBP/CBC)				
Haemoglobin	15.1	13.5 - 17.0	gm/dL	Non-Cyanide Photometric Method
Total RBC Count	5.12	4.5 - 5.5	mil/cu.mm	Electrical Impedence
Total WBC Count	8310	4000-10000	cell/cu.mm	Electrical Impedence
Platelet Count	238	150 - 410	10 ³ /ul	Electrical Impedence
Red Cell Distribution Width	14.2	11.6 - 14.0	%	Calculated
Hematocrit	47.2	40.0 - 50.0	%	Calculated
Mean Cell Volume (MCV)	92.2	83 - 101	fL	Calculated
Mean Cell Haemoglobin (MCH)	29.4	27 - 32	pg	Calculated
Mean Corpuscular Hb Conc. (MCHC)	32.0	31.5 - 34.5	gm/dL	Calculated
Neutrophils	55.4	40 - 80	%	VCSn / Microscopy
Lymphocytes	34.8	20 - 40	%	VCSn / Microscopy
Monocytes	6.7	2 - 10	%	VCSn / Microscopy
Eosinophils	2.8	1 - 6	%	VCSn / Microscopy
Basophils	0.3	0-2	%	VCSn / Microscopy
DC SUM	100.00			
Absolute Neutrophil Count	4.60	2.0 - 7.0	* 10 ⁹ /L	Calculated
Absolute Lymphocyte Count	2.89	1-3	* 10 ⁹ /L	Calculated
Absolute Monocyte Count	0.56	0.1-1.0	* 10 ⁹ /L	Calculated
Absolute Eosinophil Count	0.23	0.0-0.5	* 10 ⁹ /L	Calculated
Absolute Basophils Count	0.02	0-2	* 10 ⁹ /L	Calculated
RBC	Normocytic Normochromic			
WBC	Normal in Total Count and Differential			
Platelets	Adequate			

Reference

Fully automated haematology analyzer (Mindray BC-6000) (Colorimetry, Electrical Impedance, VCS Technology, Leishman's Stain and Microscopy). **Reference** : Dacie and Lewis Practical Hematology,12th Edition

END OF REPORT



Dr. Vishnavi Danda
Consultant Pathologist
Regd No: APMC/FMR/78761

Patient Name : MR. KOTHURU JESHWANTH**Age / Gender :** 21 years / Male**Patient ID :** 396858**Referral :** Dr. SELF**Sample Type :** Fluoride - F**Source :** DoctorC**Collection Time :** Sep 06, 2026, 03:30 p.m.**Receiving Time :** Sep 06, 2026, 05:48 p.m.**Reporting Time :** Sep 06, 2026, 07:06 p.m.**Sample ID :**

8276232

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
<u>Glucose - Fasting</u>				
Glucose fasting	76.56	Normal: 70 - 106 Impaired Tolerance: 101-125 Diabetes mellitus: ≥ 126	mg/dL	Hexikinase G6P-DH

Interpretation

A fasting blood glucose test is clinically significant because it is the most common method to screen for prediabetes and diabetes, as it measures blood sugar levels after a period of fasting, providing a reliable indicator of how well your body regulates glucose when not actively consuming food; high fasting blood glucose levels can indicate an increased risk of developing diabetes or related complications, even if symptoms aren't present.

Reference:

Beckman Series Kit insert

****END OF REPORT****

Dr. Vishnavi Danda
Consultant Pathologist
Regd No: APMC/FMR/78761

Patient Name : MR. KOTHURU JESHWANTH**Age / Gender :** 21 years / Male**Patient ID :** 396858**Referral :** Dr. SELF**Sample Type :** Serum**Source :** DoctorC**Collection Time :** Sep 06, 2026, 03:30 p.m.**Receiving Time :** Sep 06, 2026, 05:47 p.m.**Reporting Time :** Sep 06, 2026, 07:56 p.m.**Sample ID :**

8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
Iron Profile/Studies- Basic				
Iron	105.12	70 - 180 New Born : 100 - 250 Infant - 40 - 100 Children 50 - 120	µg/dL	TPTZ
UIBC	336.64	155 - 355	µg/dL	Nitroso-PSAP
TIBC	441.76	240 - 450	µg/dL	Calculated
Transferrin	309.23	176 - 280	ug/dL	Calculated
Transferrin Saturation *	23.80	20 - 50	%	calculated

Interpretation:

Disease	Iron	TIBC	UIBC	%Transferrin Saturation	Ferritin
Iron Deficiency	Low	High	High	Low	Low
Hemochromatosis	High	Low	Low	High	High
Chronic Illness	Low	Low	Low/Normal	Low	Normal/High
Hemolytic Anemia	High	Normal/Low	Low/Normal	High	High
Sideroblastic Anemia	Normal/High	Normal/Low	Low/Normal	High	High
Iron Poisoning	High	Normal	Low	High	Normal

Reference :

Tietz textbook of Clinical Chemistry, Third Edition. Carl A. Burtis and Edward R.Ashwood, eds. Philadelphia, PA: WB saunders.

Beckman Series Kit insert

****END OF REPORT******Dr. Vishnavi Danda**
Consultant Pathologist
Regd No: APMC/FMR/78761

Patient Name : MR. KOTHURU JESHWANTH

Age / Gender : 21 years / Male

Patient ID :396858

Referral : Dr. SELF

Sample Type : Serum

Source : DoctorC

Collection Time : Sep 06, 2026, 03:30 p.m.

Receiving Time : Sep 06, 2026, 05:47 p.m.

Reporting Time : Sep 06, 2026, 07:32 p.m.

Sample ID :



8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
Liver Function Test / LFT / Hepatic Panel				
Bilirubin - Total	0.59	0.3 - 1.2	mg/dL	DPD Method
Bilirubin - Direct	0.13	< 0.2	mg/dL	DPD
Bilirubin - Indirect	0.46	0.1 - 1.0	mg/dL	Serum, Calculated
Aspartate Aminotransferase (AST/SGOT)	22.2	< 50	U/L	IFCC Method with P-5-P
Alanine Transaminase (ALT/SGPT)	28.9	< 50.0	U/L	IFCC Method with P-5-P
		New born/Infant : 13 - 45		
SGOT/SGPT	0.77	0.7 - 1.4	ratio	calculated
GGT-Gamma-glutamyl transpeptidase	30	< 55.0	U/L	IFCC Method
Alkaline Phosphatase-ALPI	74.5	33 - 115	U/L	PNP-AMP
Total Protein	8.07	Adult 6.6 - 8.3	g/dL	
		Children (1-18 Y) - 5.7 - 8.0		Biuret
		New born(1-30 days)- 4.1 - 6.3		
Albumin	4.9	Adult : 3.5 - 5.2	g/dL	Biuret
		New Born (1-4 Days) - 2.8 - 4.4		
Globulin	3.17	2.0 - 3.5	g/dL	Calculated
A/G Ratio	1.55	1.2 - 2.2	ratio	Calculated

Interpretation

- LFT results reflect different aspects of the health of the liver, i.e.,

hepatocyte integrity(AST & ALT),synthesis and secretion of bile (Bilirubin ,ALP),cholestasis (ALP,GGT),protein synthesis (Albumin).

1. Hepatocellular injury:

- AST-Elevated levels can be seen. However,it is not specific to liver and can be raised in cardiac and skeletal injuries.
- ALT -Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. • Disproportionate increase in AST,ALT compared with ALP. • Bilirubin may be elevated. • AST: ALT (ratio) - In case of hepatocellular injury AST : ALT >1 In Alcoholic Liver Disease AST : ALT usually >2 This ratio is also seen to be increased in NAFLD ,Wilson's disease, Cirrhosis ,but the increase is usually not >2 2. **Cholestatic pattern:** • ALP - Disproportionate increase in ALP compared with AST,ALT. • Bilirubin may be elevated. • ALP elevation also seen in pregnancy,impacted by age and sex. • To establish the hepatic origin correlation with GGT helps. If GGT elevated indicates hepatic cause of increased ALP.

3. Synthesis function impairment:

- Albumin** - Liver disease reduces albumin levels. Correlation with PT (Prothrombin Time)

Reference

Tietz textbook of Clinical Chemistry, Third Edition. Carl A. Burtis and Edward R.Ashwood, eds. Philadelphia, PA: WB saunders.

Beckman Series Kit Insert



Dr.Vishnavi Danda
Consultant Pathologist
Regd No: APMC/FMR/78761

Patient Name : MR. KOTHURU JESHWANTH
Age / Gender : 21 years / Male
Patient ID :396858
Referral : Dr. SELF
Sample Type : Serum

Source : DoctorC
Collection Time : Sep 06, 2026, 03:30 p.m.
Receiving Time : Sep 06, 2026, 05:47 p.m.
Reporting Time : Sep 06, 2026, 07:32 p.m.
Sample ID : 
8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
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END OF REPORT





Dr. Vishnavi Danda
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Patient Name : MR. KOTHURU JESHWANTH**Age / Gender :** 21 years / Male**Patient ID :** 396858**Referral :** Dr. SELF**Sample Type :** Serum**Source :** DoctorC**Collection Time :** Sep 06, 2026, 03:30 p.m.**Receiving Time :** Sep 06, 2026, 05:47 p.m.**Reporting Time :** Sep 06, 2026, 07:28 p.m.**Sample ID :**

8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
Calcium - Serum				
Calcium*	9.06	8.8 - 10.6 0 – 10 day 7.6 – 10.4 10 day – 24 months 9.0 – 11.0 2 – 12 year 8.8 – 10.8	mg/dL	Arsenazo III

Interpretation

A calcium blood test measures the amount of calcium in your blood. Too much or too little calcium in your blood may be a sign of a wide range of medical conditions, such as bone disease, thyroid disease, parathyroid disorders, kidney disease, and other conditions.

Reference:

Tietz textbook of Clinical Chemistry, Third Edition. Carl A. Burtis and Edward R. Ashwood, eds. Philadelphia, PA: WB Saunders.

Beckman Series Kit insert

****END OF REPORT****

Dr. Vishnavi Danda
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Regd No: APMC/FMR/78761

Patient Name : MR. KOTHURU JESHWANTH

Age / Gender : 21 years / Male

Patient ID :396858

Referral : Dr. SELF

Sample Type : Blood

Source : DoctorC

Collection Time : Sep 06, 2026, 03:30 p.m.

Receiving Time : Sep 06, 2026, 05:47 p.m.

Reporting Time : Sep 06, 2026, 08:03 p.m.

Sample ID :



8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
Kidney Profile				
Blood Urea Nitrogen - BUN *	13.11	5-18 :Infant/Child 6-20: Adult 8-23: >60.0 Years	mg/dL	Calculated
Urea *	28.05	Adult 17-43 New Born 8.4 -25.8 Infant/Child 10.8-38.4	mg/dL	UV-GLDH
Uric Acid*	6.56	3.5 – 7.2	mg/dL	Enzymatic Colour Test
Creatinine*	0.9	0.67 - 1.17 Neonate -0.31 - 0.98 Infant - 0.16 - 0.39 Child -0.262 - 0.77	mg/dL	Jaffee Method-A
Urea Creatinine Ratio	31.17	- - Elevated ratio : >100.1 Reduced ratio : <40.1	mg/mg	Calculated
Sodium*	140	136 - 146	mmol/L	Indirect ISE
Potassium*	3.71	3.5 - 5.1	mmol/L	Indirect ISE
Chloride*	100	101 - 109	mmol/L	Indirect ISE
Serum Albumin	4.9	Adult - 3.5 -5.2 New Born(0 - 4day) : 2.8 -4.4	g/dL	Bromocresol Green(BCG)
BUN / Creatinine Ratio	14.57		Ratio	Calculated

Reference

Tietz textbook of Clinical Chemistry, Third Edition. Carl A. Burtis and Edward R.Ashwood, eds. Philadelphia, PA: WB saunders.

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Interpretation

- Creatinine: Muscles produce creatinine, a waste product, from creatine phosphate, a substance that stores a lot of energy. Unlike urea, the amount of creatinine generated is constant and mostly depends on muscle mass. Age, gender, race, muscularity, exercise, pregnancy, and several other physiological characteristics can all have an impact on serum creatinine levels.
- Decreased serum Creatinine is associated with increasing Age and poor muscle mass, such as muscular atrophy.
- Both acute and chronic renal disease and blockage are associated with elevated blood creatinine levels.
- Creatinine is not an appropriate indicator for identifying kidney disease in its early stages since an increase in blood creatinine is only seen when there is significant nephron damage. High Urea, Uric Acid, and Blood Urea Nitrogen (BUN) could indicate poor renal function, in addition to other etiologies
- Sodium: Low levels: prolonged vomiting or diarrhea, diminished reabsorption in the kidney, and excessive fluid retention. Pseudo-hyponatremia is a laboratory artifact. It is usually caused by hypertriglyceridemia, cholestasis (lipoprotein X), and hyperproteinemia (monoclonal gammopathy, intravenous immunoglobulin [IVIG]). Diluted sampling should also be suspected in such

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8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
cases and confirmed on a repeat fresh sample if indicated clinically. - High levels: excessive fluid loss, high salt intake, and increased kidney reabsorption. - Potassium: • Low levels: reduced intake of dietary potassium or excessive loss of potassium from the body due to diarrhea, prolonged vomiting, or increased renal excretion. - High levels: dehydration or shock, severe burns, hemolysis, diabetic ketoacidosis, and retention of potassium by the kidney. Pseudohyperkalemia, which may result from a hemolyzed or aged sample, should always be ruled out by doing a repeat electrolyte estimation on a fresh sample, as clinically indicated. - Chloride: Low levels are noted in reduced dietary intake, prolonged vomiting, and reduced renal reabsorption, as well as some forms of acidosis and alkalosis				

****END OF REPORT****

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Patient Name : MR. KOTHURU JESHWANTH

Age / Gender : 21 years / Male

Patient ID :396858

Referral : Dr. SELF

Sample Type : Serum

Source : DoctorC

Collection Time : Sep 06, 2026, 03:30 p.m.

Receiving Time : Sep 06, 2026, 05:47 p.m.

Reporting Time : Sep 06, 2026, 07:28 p.m.

Sample ID :



8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
Lipid Profile				
Cholesterol-Total	169.28	Desirable: <= 200 Borderline High: 201-239 High: > 239	mg/dL	CHE-CHO-POD
Cholesterol-HDL Direct	46.12	High Risk: < 40 Optimal: 40 - 60 Low Risk: > 60	mg/dL	Enzymatic
LDL Cholesterol	100.22	Optimal: < 100 Near / Above optimal: 100 - 129 Borderline high: 130 - 159 High: 160 - 189 Very High: >= 190	mg/dL	Calculated
Triglycerides	114.7	Normal: < 150 Borderline High: 150 - 199 High: 200 - 499 Very High: >= 500	mg/dL	GK-GPO-POD
Non - HDL Cholesterol	123.16	Desirable: < 130 Borderline High: 130 - 159 High: 160 - 189 Very High: >= 190	mg/dL	calculated
VLDL Cholesterol	22.94	10 - 30	mg/dL	calculated
CHOL/HDL RATIO	3.67	0.0 - 5.0	ratio	calculated
LDL/HDL RATIO	2.17	0.0 - 3.5	ratio	calculated
HDL/LDL RATIO	0.46	0.0 - 3.5	ratio	calculated

Interpretation

Interpretation:

- For non-fasting samples, the biological reference interval remains the same for all parameters, except for triglyceride as cholesterol (HDL, LDL, total), which changes only by a small amount in the non-fasting state; the recommended desired value for triglycerides is 200 mg/dl, are recommended to perform a follow-up fasting lipid panel in 2 to 4 weeks.
- As per the consensus of the Lipid Association of India, Non-HDL cholesterol and LDL cholesterol can be used as targets to monitor the effectiveness of lipid-lowering therapy.

Associated tests: Apolipoproteins A1, Apolipoproteins B, Apolipoprotein B/A1 Ratio, Lipoprotein(a)

Reference :

Dr. Vishnavi Danda
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8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
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Tietz textbook of Clinical Chemistry, Third Edition. Carl A. Burtis and Edward R. Ashwood, eds. Philadelphia, PA: WB Saunders.

Beckman AU Series Kit insert

****END OF REPORT******Dr. Vishnavi Danda**
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Patient Name : MR. KOTHURU JESHWANTH**Age / Gender :** 21 years / Male**Patient ID :** 396858**Referral :** Dr. SELF**Sample Type :** Serum**Source :** DoctorC**Collection Time :** Sep 06, 2026, 03:30 p.m.**Receiving Time :** Sep 06, 2026, 05:47 p.m.**Reporting Time :** Sep 06, 2026, 07:08 p.m.**Sample ID :**

8276224

BIOCHEMISTRY

Test Description	Value(s)	Biological Reference Intervals	Unit(s)	Methodology
Thyroid Profile Test				
T3(TRI-IODOTHYRONINE)	1.37	0.87 - 1.78	ng/ml	CLIA
T4(THYROXNE)	9.4	Adults : 4.82 - 15.65 Birth - 14 days : 11.8 - 22.6 14 days - 5 yrs : 7.2 - 16.6 5 - 15 yrs : 6.4 - 13.3	µg/dL	CLIA
TSH(THYROID STIMULATING HORMONE)	3.819	Adult Male : 0.45 - 5.33	µIU/mL	CLIA

Interpretation :

- Assay results should be interpreted in context to the clinical condition and associated results of other investigations.
- Previous treatment with corticosteroid therapy may result in lower TSH levels while Thyroid hormone levels are normal.
- Results are invalidated if the client has undergone a radionuclide scan within 7-14 days before the test.
- Abnormal thyroid test findings often found in critically ill clients should be repeated after the critical nature of the condition is resolved.
- The production, circulation, and disposal of Thyroid hormone are altered throughout the stages of pregnancy. Hyperthyroidism (overactive thyroid): Hyperthyroidism (overactive Thyroid) occurs when your thyroid gland produces too much of the hormone Thyroxine.
- Hyperthyroidism can accelerate your bodys metabolism, causing unintentional weight loss and a rapid or irregular heartbeat.
- Hypothyroidism (underactive thyroid): Hypothyroidism (underactive thyroid) is a condition in which your Thyroid gland doesnt produce enough of certain crucial hormones.
- Hypothyroidism may not cause noticeable symptoms in the early stages.
- Over time, untreated Hypothyroidism can cause a number of health problems, such as obesity, joint pain, infertility and heart disease.

Reference: Beckman Coulter DXI800 Kit Insert

END OF REPORT

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